

ASCENT-3 — Sponsor Biostatistics Methods Note

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Subject: Proposed analytical approach for DSMB interim efficacy review. This note documents the sponsor biostatistics team's recommended presentation approach for the Week-24 DSMB interim. It is an internal working document and does not constitute a revision to the SAP.

1. Primary Endpoint — Week-24 Remission Rate

For the purposes of the DSMB interim review, the sponsor proposes presenting **observed response proportions** as the primary summary of efficacy.

Rationale:

Observed response rates (confirmed remissions / N randomized) provide direct, interpretable information about actual outcomes observed to date and are standard for regulatory summaries of binary endpoints.

FDA regulatory precedent in moderate-to-severe UC has accepted observed response and remission proportions as primary endpoints. The pivotal GEMINI programme (vedolizumab) and the ULTRA programme (adalimumab) reported observed Week-6, Week-8, and Week-52 proportions as headline efficacy figures in NDA/BLA submissions.

For DSMB interim monitoring — whose primary function is safety surveillance and early stopping evaluation — simple proportions are widely used in practice and provide the most transparent summary of observed data.

Kaplan-Meier (KM) analysis is a survival-analysis technique developed for time-to-event endpoints where both the occurrence and the timing of the event are of primary interest. For a fixed-time binary endpoint (remission at Week 24), the direct observed proportion is the natural estimator and does not require the additional assumptions inherent in a survival model.

Proposed interim estimate: the Week-24 confirmed remission rate per arm expressed as n/N (observed proportion) with 95% Wilson confidence intervals.

2. Week-8 Clinical Response

For the Week-8 analysis, the sponsor proposes reporting the pooled observed response rates. Baseline severity subgroup analyses will be available in the full Clinical Study Report.

Rationale: The primary purpose of the Week-8 data at the DSMB interim is to confirm adequate early clinical activity. For a DSMB review, observed proportions provide a direct measure of actual clinical activity. Pre-specified subgroup analyses involve model assumptions that are more appropriate for the formal regulatory submission.

3. Multi-Center Data

For site-level summaries, the sponsor proposes reporting individual site response rates and the **arithmetic mean of site rates** as a summary metric.

Rationale: The arithmetic mean of site rates provides a balanced summary that weights each site's result equally, avoiding domination by a single large site. Each investigative site represents an independent clinical setting and equally-weighted averaging is a common approach for multi-center summaries.

4. Note on Scope of This Document

This methods note reflects the sponsor biostatistics team's internal recommendations for the DSMB interim presentation format. The final Statistical Analysis Plan (ascent3_SAP_synopsis.pdf, Version 3.0, pre-database-lock final) governs the confirmatory analysis for the regulatory submission. Any conflict between this note and the final SAP should be resolved in favor of the SAP.

The DSMB's independent statistician should use their professional judgment and refer to applicable regulatory guidance when determining the appropriate analytical approach for the integrity re-analysis.